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SURGICAL PATHOLOGY REPORT
*** ADDENDUM REPORT ***

ADDENDUM REPORT NUMBER TWO
BREAST PROGNOSTIC PANEL: Block B2

TEST	RESULT	REFERENCE RANGES
DNA Analysis by Image: DNA Index:	1.1 (DIPLOID)	0.9 - 1.1 is Diploid < 0.9 or > 1.1 is Aneuploid

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	KMT	6/10/11
Date Reviewed	6/10/11	6/13/11

Her2 Gene Amplification By FISH: UNABLE TO BE DETERMINED

Average copies per cell:
Her-2/neu:
Chromosome 17:
Ratio: **> 2.0 Her2/neu**

COMMENT: This specimen failed to hybridize successfully with probes specific to Her2/neu gene and chromosome 17 alpha satellite sequences. Non formalin and ethanol based fixatives as well as age and storage conditions of the specimen may contribute to unsuccessful FISH analysis. Repeat studies are recommended if additional tissue becomes available from subsequent specimens.

Case Interpreted by:

Notes:

- *Quantitative analysis performed using Chromavision ACIS.
- The Her2 /neu and CEP17 probes are manufactured by ' '.
- The Her2/neu gene detection by Fluorescence In Situ Hybridization (FISH) was performed using the LSI Her2/neu DNA probe, specific for the Her2/neu gene locus 17q11.2-q12 and the CEP1 DNA probe specific for the alpha satellite DNA sequence at the centromeric region of chromosome 17 (17p11.1-q11.1)
- These tests were developed and performance characteristics determined by the **They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such approval or clearance is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Act (CLIA) of 1988 as qualified to perform high complexity clinical laboratory testing.**

Addendum Report Issued By:

ADDENDUM REPORT NUMBER ONE
BREAST PROGNOSTIC PANEL: (Preliminary test results on block B2.)

TEST	RESULT	REFERENCE RANGES
Estrogen Receptor:	Positive (95%)	> 4% is Positive 2-4% is Borderline < 2% is Negative
Progesterone Receptor:	Negative (0%)	> 4% is Positive 2-4% is Borderline < 2% is Negative
KI-67 (MIB1) Proliferation Marker:	Low (<10%)	> 20% is High 10-20% is Borderline < 10% is Low

ICD-0-3

carcinoma, infiltrating
lobular, NOS 8520/3

Site: breast, NOS C50.9

fw 6/10/11

These results were interpreted by [REDACTED] An additional addendum report will follow when DNA and Her-2-neu tests are completed.

These tests were developed and performance characteristics determined by [REDACTED] They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such approval or clearance is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Act (CLIA) of 1988 as qualified to perform high complexity clinical laboratory testing

Addendum Report Issued By:
[REDACTED]

DIAGNOSIS

DIAGNOSIS:

A,B,C: Left mastectomy with left sentinel lymph node and additional lymph node for diagnosis of breast carcinoma:

Tumor Characteristics:

1. Histologic type: Infiltrating lobular carcinoma.
 2. Size: 2.5 x 2 x 1.8 cm.
 3. Elston modification of Bloom-Richardson grade:
 - a. Architectural Score: 3/3.
 - b. Nuclear Score: 2/3.
 - c. Mitotic Score: 1/3.
 - d. Total score: 6/9 = Grade: II.
 4. In-situ component: Present.
 - e. Amount: Less than 25%.
 - f. Histologic type: Lobular carcinoma in situ.
 - g. Architectural type: Solid.
 - h. Nuclear grade: II.
 - i. Comedo necrosis: Absent.
 5. Margin status for in situ carcinoma: Not transected.
 6. Lymphovascular space invasion: No.
 7. Skin involvement: No.
 8. Are microcalcifications identified in association with tumor: No.
- Surgical Margin Status:
1. Is tumor transected: No.
 2. Tumor distance to closest margin: 1.5 cm deep margin.
- Lymph Node Status:
1. Total number of lymph nodes examined: Ten.
 2. Total number of lymph nodes containing metastatic carcinoma by gross exam or light microscopy: Sentinel, 1/2; Level I and II, 1/8.
 3. Size of largest metastasis: 9 mm.
 4. Extra-capsular extension of tumor: No.
- Other:
1. Other significant findings: Fibrocystic change with epithelial hyperplasia and benign calcifications.
 2. Block prognostic indicators will be obtained on paraffin block B2.
 3. Correlation statement with intra-op consult or frozen section: Frozen section diagnosis sentinel lymph is confirmed by permanent section analysis.
 4. pTNM stage: T2N1MX
 5. If further testing is required in the future, infiltrating carcinoma is best seen in blocks B2,3,4 and 5.

DIAGNOSIS

D. Skin, lip, biopsy:

Benign solar lentigo (pigmented actinic keratosis).

Electronic Signature:

CLINICAL INFORMATION

CLINICAL HISTORY:

Preoperative Diagnosis: Left breast cancer

Postoperative Diagnosis:

Symptoms/Radiologic Findings:

SPECIMENS:

- A. Left sentinel lymph node FS
- B. Left breast tissue
- C. Left lymph nodes
- D. Seborrheic keratosis of lip

SPECIMEN DATA

GROSS DESCRIPTION:

- A. Container A is labeled [redacted], A and consists of a tan-yellow firm fatty nodule, 2.0 x 1.0 x 1.0 cm in greatest dimension. The specimen is trisected and entirely submitted for frozen section in one cassette labeled [redacted].
- B. Container B is labeled [redacted], B and consists of a 265 gram portion of left breast tissue, 14.0 x 11.0 x 6.0 cm with attached tan-gray ellipse of skin, 5.0 x 4.2 cm displaying a centrally located flattened nipple, 0.8 cm in greatest dimension. The deep margin is inked and the specimen is sectioned to reveal predominately tan-yellow lobular adipose tissue intermixed with moderate amounts of tan-gray dense fibrous tissue. A gray-white stellate gritty mass is noted in the upper outer quadrant and is 2.5 x 2.0 x 1.8 cm in greatest dimension. Samples of this mass have been submitted for genomic studies. This mass is located 2.0 cm from the skin surface and 1.5 cm from the deep margin. No other lesions are grossly noted. The lateral margin displays a gray-white minute nodule possibly representing lymph node, 0.2 cm. Representative sections submitted in eleven cassettes labeled [redacted] as follows: nipple-1; stellate mass-2 to 5; deep margin-6; upper inner quadrant-7; upper outer-8; lower inner-9; lower outer-10; lateral nodule-11.
- C. Container C is labeled [redacted], C consists of a tan-yellow lobular mass of adipose tissue, 11.0 x 4.0 x 2.0 cm in greatest dimension. Sectioning reveals eight tan-yellow firm fatty nodules, the largest 1.5 cm. Samples of these nodules submitted in four cassettes labeled [redacted].
- D. Container D is labeled [redacted], D and consists of a tan-gray granular raised lesion, 0.5 x 0.3 x 0.2 cm. The skin surface displays a mottled dark brown discoloration. The specimen is inked, serially sectioned and entirely submitted in one cassette labeled 1 [redacted].